

1. Executive Summary

AbbVie's public Medicare exposure is no longer just a Part D story. As of April 9, 2026, the company has one selected Part B and Part D brand in the 2028 negotiation cycle, two major Part D brands with negotiated prices already set for 2027, and one major Part D oncology brand with a 2026 negotiated price already in effect. The public AbbVie 2025 Form 10-K confirms the negotiation status of Imbruvica, Vraylar, Linzess, and Botox, while CMS's 2026 and 2027 fact sheets provide the negotiated pricing and selection dates. The implication is operational, not abstract: AbbVie needs a reimbursement leader who can manage ASP/payment-limit mechanics, Part D formulary friction, and CMS-facing policy execution as one portfolio problem, not as separate brand issues. AbbVie 2025 Form 10-K (<https://www.sec.gov/Archives/edgar/data/1551152/000155115226000008/abbv-20251231.htm>), CMS 2026 negotiated prices (<https://www.cms.gov/files/document/fact-sheet-negotiated-prices-initial-price-applicability-year-2026.pdf>), CMS 2027 negotiated prices (<https://www.cms.gov/files/document/fact-sheet-negotiated-prices-ipay-2027.pdf>), CMS 2028 selected drugs (<https://www.cms.gov/files/document/factsheet-medicare-negotiation-selected-drug-list-ipay-2028.pdf>)

The most important Part B brands for this seat are Botox, Skyrizi IV induction, and Elahere. The current CMS April 2026 Medicare Part B payment-limit file shows J0585 at \$6.512 per unit, J2327 at \$14.599 per mg, and J9063 at \$71.128 per mg. AbbVie's 2025 Form 10-K shows Botox Therapeutic U.S. net revenues of \$3.151 billion, Skyrizi U.S. net revenues of \$15.202 billion, and Elahere U.S. net revenues of \$607 million in 2025, so the reimbursement consequences are material. CMS April 2026 payment-limit file (<https://www.cms.gov/files/zip/april-2026-medicare-part-b-payment-limit-files-03-30-2026-final-file.zip>), AbbVie 2025 Form 10-K (<https://www.sec.gov/Archives/edgar/data/1551152/000155115226000008/abbv-20251231.htm>)

The job description maps to a real set of workstreams: CMS engagement, coding and ASP monitoring, label-change reimbursement planning, discarded-drug policy management, launch support, provider-society engagement, and translation of IRA policy into brand-level tactics. The public evidence says this role must be fluent in both benefits because AbbVie's portfolio now spans provider-administered Part B products, self-administered Part D products, and mixed/transition products such as Skyrizi. That is the core commercial interpretation from the sources. CMS Part B drugs overview (<https://www.cms.gov/cms-guide-medical-technology-companies-and-other-interested-parties/payment/part-b-drugs>), AbbVie 2025 Form 10-K (<https://www.sec.gov/Archives/edgar/data/1551152/000155115226000008/abbv-20251231.htm>)

Joseph Stewart is a plausible fit because the workspace materials show direct reimbursement, policy, PBM/formulary, and manufacturer-facing consulting experience, plus prior Otsuka and biosimilars policy work. The bridge is strongest where the role needs Medicare fluency, access strategy, payer/provider translation, and launch readiness. The bridge is weaker where AbbVie-specific Part B buy-and-bill depth is expected; that should be addressed honestly as a transfer of skills, not a claim of identical prior ownership. About page ([app/about/page.tsx](#)), Portfolio master ([joseph_market_access_portfolio_master.md](#)), Portfolio brief ([portfolio_brief.md](#))

Evidence labels used below

- **Confirmed fact** means directly supported by a primary or reputable public source.
- **Inference** means a conclusion drawn from the public record.
- **Commercial interpretation** means the business implication of the confirmed facts.
- **Unavailable from reliable public source** means I could not validate the number or claim without a subscription or internal source.

2. What This Role Actually Owns

Posting responsibility	What it means in practice	Likely internal stakeholders	Likely external stakeholders	Reimbursement artifacts this role would produce
Part B reimbursement strategy	Build the company's payment strategy around ASP, payment-limit files, coding, claims edits, discarded-drug rules, site-of-care economics, and product-specific reimbursement friction.	Government affairs, market access, HEOR, legal, finance, brand teams, field reimbursement, launch teams.	CMS, MACs, provider groups, specialty pharmacies, hospital systems, trade associations.	Product reimbursement strategy memos, ASP impact models, coding guides, claim submission guidance, CMS comment letters.
CMS engagement	Shape CMS policy interpretation and implementation before and after rulemaking.	Federal policy, legal, advocacy, leadership, brand owners.	CMS, HHS, congressional staff, provider societies.	Meeting prep, comment letters, data submissions, talking points, issue briefs.
Product-specific advocacy	Translate enterprise policy positions into brand-by-brand positions by indication and channel.	Brand teams, medical affairs, launch leads, payer strategy.	Specialty societies, MACs, payer policy teams, hospital revenue-cycle teams.	Brand dossiers, reimbursement FAQs, HCP billing decks, society-facing issue summaries.
Launch planning support	Make sure a new or label-expanded product is reimbursable, billable, and operationally usable at launch.	Launch excellence, medical, regulatory, supply chain, finance, market access.	Providers, distributors, specialty pharmacies, payers, coding consultants.	Launch reimbursement checklist, coding launch plan, site-of-care model, reimbursement timeline.
Label change reimbursement planning	Reassess payment, coding, and coverage when a label expands or changes route, dose, or indication.	Regulatory, medical, brand, field access, pricing.	CMS, MACs, payers, provider groups.	Label-change impact memo, coding-change tracker, payer notification strategy.
Provider-society engagement	Influence specialty practice behavior where coding, administration, and payment rules create or reduce friction.	Medical affairs, field reimbursement, brand leads.	Specialty societies, clinician leaders, practice administrators.	Society decks, education plans, coding updates, advocacy agendas.
Policy monitoring with product lens	Track federal policy and turn it into product-level action, not a general policy report.	Corporate policy, brand teams, legal, finance.	CMS, Capitol Hill, trade groups, provider organizations.	Weekly issue tracker, product exposure matrix, policy risk register.
Commercial impact translation	Convert policy change into net revenue, access, and adoption consequences.	Finance, brand leadership, commercial operations.	Payers, GPOs, providers, pharmacy benefit managers.	Impact models, payer scenario analyses, leadership slides, decision memos.
Enterprise-to-brand alignment	Create one reimbursement narrative across brands with very different channels and Medicare benefits.	Portfolio strategy, enterprise leadership, brand executives.	External stakeholders as needed.	Portfolio heat map, negotiation playbook, enterprise policy doctrine.

The key point is that this is not a passive monitoring role. It is a control-tower role that sits between CMS policy and brand execution. That reading is grounded in the job description you provided and in AbbVie's public portfolio mix, especially the contrast between Part B provider-administered brands and Part D oral brands. AbbVie 2025 Form 10-K (<https://www.sec.gov/Archives/edgar/data/1551152/000155115226000008/abbv-20251231.htm>)

3. AbbVie Medicare Portfolio Map

Product	Therapeutic area	Route of administration	Usual benefit orientation	Site of care relevance	Provider-administered	Buy-and-bill relevance	Key reimbursement friction points	IRA negotiation status	Notes
Botox / Botox Cosmetic	Neuroscience, aesthetics, movement disorders, migraine	Injectable	Part B for therapeutic uses; Part D only as selected in IPAY 2028 framework because CMS selected Botox/Botox Cosmetic for Parts B and D	Office, clinic, outpatient	Yes	Yes	HCPCS/J-code, single-dose waste, dosing variation by indication, ASP/payment-limit changes, Part B negotiation	Selected for IPAY 2028	Confirmed fact: selected on January 27, 2026. Commercial interpretation: most visible AbbVie Part B policy exposure.
Skyrizi	Immunology	IV induction plus SC maintenance	Mixed / cross-benefit	Infusion center for induction; specialty pharmacy or self-admin for maintenance	IV induction yes; SC maintenance no	IV induction yes	Benefit transition, J2327, site-of-care, prior auth, channel handoff, Medicare plan control	Not selected	Confirmed fact: IV induction is single-dose vial; SC forms are plan-managed in Part D-like settings.
Rinvoq	Immunology	Oral tablet	Part D	Home / specialty pharmacy	No	No	Tiering, PA, step therapy, quantity limits, indication-based coverage	Not selected	High commercial importance and likely future structural risk.
Humira	Immunology	SC injection	Part D residual / declining	Home / specialty pharmacy	No	No	Biosimilar competition, formulary pressure, residual Medicare access, contract erosion	Not selected	Commercial pressure remains, but biosimilar competition reduces negotiation attractiveness.
Vraylar	Neuroscience	Oral capsule	Part D	Home / specialty pharmacy	No	No	Tiering, PA, quantity limits, restricted access	Selected for IPAY 2027	Confirmed fact: AbbVie/Part D negotiated price effective January 1, 2027.
Linzess	Gastroenterology	Oral capsule	Part D	Home / retail or specialty pharmacy	No	No	Tiering, PA, quantity limits, step management	Selected for IPAY 2027	Confirmed fact: AbbVie/Part D negotiated price effective January 1, 2027.
Imbruvica	Oncology	Oral tablet/capsule	Part D	Home / specialty pharmacy	No	No	Tiering, PA, quantity limits, oncology benefit management	Selected for IPAY 2026	Confirmed fact: AbbVie/Part D negotiated price effective January 1, 2026.
Venclexta	Oncology	Oral tablet	Part D	Home / specialty pharmacy, but regimen economics matter in clinic	No	No	Tiering, PA, quantity limits, regimen-level economics	Not selected	High oncology spend and ongoing structural risk, but not yet selected.

Product	Therapeutic area	Route of administration	Usual benefit orientation	Site of care relevance	Provider-administered	Buy-and-bill relevance	Key reimbursement friction points	IRA negotiation status	Notes
Elahere	Oncology	IV infusion	Part B	Infusion center / HOPD / office	Yes	Yes	J9063, single-dose waste, site-of-care, dosing by weight, ASP/payment-limit exposure	Not selected	Confirmed fact: permanent J-code and single-dose vial.
Qulipta	Migraine	Oral tablet	Part D	Home / retail or specialty pharmacy	No	No	Tiering, PA, quantity limits	Not selected	Public plan evidence shows controlled formulary positioning.
Ubrovelvy	Migraine	Oral tablet	Part D	Home / retail or specialty pharmacy	No	No	Tiering, PA, quantity limits	Not selected	Public plan evidence shows controlled formulary positioning.
Vyalev	Parkinson's disease	Continuous subcutaneous infusion	Mixed / operationally complex	Specialty pharmacy / infusion-adjacent	Usually yes, but benefit classification is product-specific and can be operationally mixed	Inference	Device-adjacent handling, pump workflow, specialty channel friction	Not selected	Public evidence supports provider-managed complexity; benefit classification should be verified before any claims.
Duopa / Duodopa	Advanced Parkinson's disease	Enteral suspension	Mixed / operationally complex	Procedure- and device-adjacent	Usually yes, but benefit classification is product-specific and can be operationally mixed	Inference	Pump placement, site-of-care, device workflow, Medicare payment alignment	Not selected	Public evidence supports site-of-care complexity.
Ozurdex	Retina / eye care	Intravitreal implant	Part B	Ophthalmology / clinic	Yes	Yes	Coding, administration, site-of-care, ASP/payment-limit visibility	Not selected	Smaller commercial footprint, but clearly relevant to Part B policy monitoring.

AbbVie's 2025 Form 10-K confirms the broad revenue mix behind this map: Skyrizi, Rinvoq, Humira, Vraylar, Botox Therapeutic, Ubrovelvy, Qulipta, Vyalev, Duodopa, Imbruvica, Venclexta, Elahere, Linzess/Constella, Mavyret, Creon, and Ozurdex all appear in the company's product revenue table. The company also explicitly states that Imbruvica, Vraylar, Linzess, and Botox have been selected under the Medicare Drug Price Negotiation Program. AbbVie 2025 Form 10-K (<https://www.sec.gov/Archives/edgar/data/1551152/000155115226000008/abbv-20251231.htm>)

4. AbbVie Products Already Subject to IRA Negotiation

Product	AbbVie relationship	Benefit	IPAY year	Selection date	Relevant CMS fact sheet	Negotiated price if public	Comparator list price if public	Discount if public	Operational consequence for AbbVie	Notes
Imbruvica	AbbVie / Pharmacyclics collaboration	Part D	2026	August 29, 2023	IPAY 2026 negotiated prices (https://www.cms.gov/files/document/fact-sheet-negotiated-prices-initial-price-applicability-year-2026.pdf)	\$9,319 for a 30-day supply	\$14,934 WAC proxy for 30-day supply	38%	Lower Medicare net, formulary and gross-to-net pressure, downstream channel strategy implications	Confirmed fact from CMS and AbbVie 10-K.
Vraylar	Direct AbbVie	Part D	2027	January 17, 2025	IPAY 2027 negotiated prices (https://www.cms.gov/files/document/fact-sheet-negotiated-prices-ipay-2027.pdf)	\$770 for a 30-day supply	\$1,376 WAC proxy for 30-day supply	44%	Lower Medicare net, tighter Part D access management, reference point for future negotiation doctrine	Confirmed fact from CMS and AbbVie 10-K.
Linzess	Direct AbbVie	Part D	2027	January 17, 2025	IPAY 2027 negotiated prices (https://www.cms.gov/files/document/fact-sheet-negotiated-prices-ipay-2027.pdf)	\$136 for a 30-day supply	\$539 WAC proxy for 30-day supply	75%	Large discount, clear Part D formulary and access implications	Confirmed fact from CMS and AbbVie 10-K.
Botox / Botox Cosmetic	Direct AbbVie	Part B and Part D selected for negotiation	2028	January 27, 2026	IPAY 2028 selected-drug list (https://www.cms.gov/files/document/factsheet-medical-re-negotiation-selected-drug-list-ipay-2028.pdf)	Not yet public	Not yet public	Not yet public	Requires Part B reimbursement planning before MFP publication, because payment-limit and ASP mechanics will matter on day one	Confirmed selection; negotiated price not yet public.

Future structural negotiation risk

Product	Why it is at risk	Evidence basis	Confidence
Rinvoq	High Medicare-relevant spend, oral small-molecule profile, and broad chronic use make it a logical future candidate once statutory age and ranking thresholds are met.	Inference from AbbVie revenue profile and FDA/portfolio age; not yet selected.	Medium
Ubrelvy	High-growth oral migraine brand with Medicare relevance and an oral small-molecule profile.	Inference from current Part D formulary visibility and product age; not yet selected.	Medium
Qulipta	Same logic as Ubrelvy: oral small molecule, Medicare relevance, growing utilization, and likely future spend concentration if growth continues.	Inference from current Part D formulary visibility and product age; not yet selected.	Medium
Venclexta	Oral oncology brand with real Medicare spend and long enough commercial maturity to remain a structural candidate in future cycles.	Inference from AbbVie revenue table and oncology treatment role; not yet selected.	Medium
Skyrizi	Large and growing spend, but biologic timing pushes risk further out than small molecules.	Inference from AbbVie revenue table and route; not yet selected.	Low to medium
Elahere	Provider-administered oncology brand with Medicare relevance and a meaningful ASP footprint.	Inference from route, coding, and spend; not yet selected.	Low to medium

The important distinction is between confirmed selection and structural risk. Botox is confirmed selected for IPAY 2028; the others above are inference-based future candidates because they combine Medicare relevance, commercial scale, and statutory eligibility over time. I am not treating those as confirmed CMS selections. CMS 2028 selected-drug list (<https://www.cms.gov/files/document/factsheet-medicare-negotiation-selected-drug-list-ipay-2028.pdf>), AbbVie 2025 Form 10-K (<https://www.sec.gov/Archives/edgar/data/1551152/000155115226000008/abbv-20251231.htm>)

5. Part B Negotiation Timeline and Operational Implications

Milestone	Date	What happened / will happen	Why it matters to AbbVie
Final guidance for IPAY 2028	September 30, 2025	CMS issued final guidance for the third cycle.	Sets the negotiation rules and implementation calendar.
Selected-drug announcement	January 27, 2026	CMS announced the 15 drugs selected for the third cycle, including Botox/Botox Cosmetic.	Starts the Botox war room and the Part B/B-benefit planning cycle.
Participation agreement deadline	February 28, 2026	Manufacturers had a statutory deadline to elect participation.	Confirms who is inside the negotiation process.
Data submission deadline	March 1, 2026	Participating manufacturers and the public could submit data and information.	This is the first real evidence-submission milestone.
Public engagement	Spring 2026	CMS plans patient-focused roundtables and town halls in spring 2026.	The advocacy narrative has to be ready before these meetings.
Initial offer	No later than June 1, 2026	CMS will send the initial offer with a concise justification.	This is where price discipline becomes concrete.
Counteroffer window	30 days after initial offer	The manufacturer can accept or counter.	Reimbursement, legal, and commercial teams need pre-baked response options.
Additional negotiation meetings	Summer and Fall 2026	CMS can conduct up to two more negotiation meetings.	Escalation point for policy, economics, and access arguments.
Negotiation period end	November 1, 2026	Negotiation period ends.	Locks in the final federal price outcome for IPAY 2028.
MFP publication	By November 30, 2026	CMS will publish agreed MFPs and explanations.	Launches provider messaging, ASP/payment-limit planning, and net impact modeling.
MFP explanation deadline	No later than March 1, 2027	CMS must publish MFP explanations by statute.	Allows providers, plans, and trade groups to understand the CMS rationale.
Effective date	January 1, 2028	Negotiated prices become effective.	The reimbursement change becomes operational, not theoretical.

Operational meaning for a Part B reimbursement director

Confirmed fact: CMS's April 2026 ASP Pricing Files page says Medicare generally publishes Part B payment limits quarterly and that claims can still be processed locally when a payment limit is not listed. CMS guidance on IPAY 2028 and MFP effectuation shows that negotiated-price implementation has to be coordinated with the existing ASP/payment-limit framework. ASP Pricing Files (<https://www.cms.gov/medicare/payment/part-b-drugs/asp-pricing-files>), IPAY 2028 final guidance (<https://www.cms.gov/files/document/ipay-2028-final-guidance.pdf>)

Commercial interpretation: once a Part B AbbVie product is selected, the issue is not only the negotiated price itself. The issue is how that price interacts with ASP reporting, quarterly payment-limit files, provider reimbursement, charge capture, waste management, and field messaging. That is exactly why a Director of Government Affairs - U.S. Reimbursement role is operationally different from a pure policy role.

Implication for Botox: the brand is multi-indication, high-volume, and supplied in single-dose vials. The public FDA label says BOTOX is supplied in 100 Unit and 200 Unit vials and is single-dose only, which makes discarded-drug compliance, JW/JZ behavior, and site-level waste management especially relevant. CMS's discarded-drug policy says JZ is required when there are no discarded amounts from single-dose containers or single-use packages, and JW/JZ compliance affects claim correctness. BOTOX label (https://www.accessdata.fda.gov/drugsatfda_docs/label/2023/103000s5327lbl.pdf), CMS discarded-drug article (<https://www.cms.gov/medicare-coverage-database/view/article.aspx?CptHcpcsCode=j1561&articleId=52855>)

Implication for the role: a reimbursement director at AbbVie would need a standing model that answers four questions at the same time: what the negotiated price is, what CMS payment-limit files will do to ASP-based reimbursement, how provider economics change under the new price, and how to message the change to physician offices, hospitals, and societies without creating operational confusion.

6. Product-Level Coding, ASP, Pricing, and Payment Evidence

Product	HCPCS / J-code	Billing unit definition	Package / vial / presentation	Single-dose or multi-dose	Discarded-drug relevance	Current ASP payment limit source	Current payment amount	Label dosing anchor	Site-of-care notes	Source link
Botox	J0585	1 UNIT	BOTOX 100 Unit and 200 Unit vials	Single-dose only	High	CMS April 2026 Medicare Part B Payment Limit File	\$6.512 per unit	Therapeutic labeling is indication-specific; vial sizes are 100 and 200 Units	Office-based injection, multiple indications, high waste-management sensitivity	CMS file (https://www.cms.gov/files/zip/april-2026-medicare-part-b-payment-limit-files-03-30-2026-final-file.zip), FDA label (https://www.accessdata.fda.gov/drugsatfda_docs/label/2023/103000s5327lbl.pdf)
Skyrizi IV induction	J2327	1 MG	600 mg/10 mL single-dose vial for IV use; SC forms are separate	Single-dose vial for IV product	High	CMS April 2026 Medicare Part B Payment Limit File	\$14.599 per mg	IV induction uses 600 mg/10 mL vial; label shows IV infusion dosing in Crohn's disease and ulcerative colitis	Infusion-center and HOPD economics; benefit transition to SC maintenance	CMS file (https://www.cms.gov/files/zip/april-2026-medicare-part-b-payment-limit-files-03-30-2026-final-file.zip), FDA label (https://www.accessdata.fda.gov/drugsatfda_docs/label/2024/761105s032lbl.pdf)
Elahere	J9063	1 MG	100 mg/20 mL single-dose vial	Single-dose	High	CMS April 2026 Medicare Part B Payment Limit File	\$71.128 per mg	6 mg/kg AIBW every 3 weeks	Classic buy-and-bill oncology economics; site-of-care and waste matter	CMS file (https://www.cms.gov/files/zip/april-2026-medicare-part-b-payment-limit-files-03-30-2026-final-file.zip), FDA label (https://www.accessdata.fda.gov/drugsatfda_docs/label/2022/761310s000lbl.pdf)

Pricing and payment discipline

- **Confirmed fact:** the payment amounts above come from the CMS April 2026 Medicare Part B Payment Limit File, effective April 1, 2026 through June 30, 2026, and are based on 4Q25 ASP data. CMS file (<https://www.cms.gov/files/zip/april-2026-medicare-part-b-payment-limit-files-03-30-2026-final-file.zip>)
- **Confirmed fact:** CMS says Medicare pays most separately payable Part B drugs at ASP plus 6% and that WAC can be used when ASP data are unavailable. CMS Part B drugs overview (<https://www.cms.gov/cms-guide-medical-technology-companies-and-other-interested-parties/payment/part-b-drugs>)
- **Unavailable from reliable public source:** current public WAC or list-price proxy for Botox, Skyrizi IV, and Elahere beyond what is already visible in negotiated-price or product-label context.
- **Commercial interpretation:** for Botox and Elahere, single-dose vials and indication-specific dosing make discard management and claims accuracy core reimbursement issues, not back-office details.

7. Part D Tiering and Formulary Positioning Evidence

Method

I used a current public Medicare Advantage Part D formulary sample from MetroPlusHealth, dated January 20, 2026 / assembled January 2026 for CY 2026, because it provides explicit tiering plus prior authorization, quantity limit, and step-therapy flags. This is not a national average. It is a proof-of-concept current plan-level snapshot from a public Medicare formularies source. MetroPlus CY 2026 formulary (https://metroplus.org/wp-content/uploads/2026/01/H0423_MEM26_3567_C-09192025-MPH-formulary_Advantage_assembled_01202026.pdf)

Product	Sample plan-level evidence	What it shows	Interpretation
Vraylar	Tier 5; PA	Strong restriction	Medicare Part D plans can place AbbVie neuroscience brands in the highest control tier.
Linzess	Tier 3; QL (16 tabs / 30 days), PA	Moderate control	Residual access remains, but controls are real.
Imbruvica	Tier 5; QL; NM; PA	Strong restriction	Negotiated brands still face active formulary management.
Venclexta	Tier 3 for 10 mg, Tier 5 for 50 mg and 100 mg; QL; NM; PA	Mixed tiering by strength	Plan-level controls can vary by strength and presentation.
Humira	Tier 5; QL; NM; PA	Strong restriction	Residual Medicare business is still highly managed.
Rinvoq	Tier 5; QL; NM; PA	Strong restriction	AbbVie's core growth immunology oral brand is heavily managed.
SKYRIZI IV / maintenance forms	Tier 5; QL; NM; PA	Strong restriction	Both Part B-adjacent and self-administered forms can face tight management.
Qulipta	Tier 3; QL; PA	Moderate control	A newer migraine brand can still be accessible but closely managed.
Ubrovelvy	Tier 3; QL; PA	Moderate control	Another migraine brand with controlled access rather than open access.

The same public formulary also shows the legend: PA means prior authorization, QL means quantity limits, ST means step therapy, NM means not available at mail-order, and B/D means covered under Medicare B or D. That matters because the operating burden is not just whether a drug is covered. It is how hard it is to get, dispense, and refill it. MetroPlus CY 2026 formulary (https://metroplus.org/wp-content/uploads/2026/01/H0423_MEM26_3567_C-09192025-MPH-formulary_Advantage_assembled_01202026.pdf)

What this says about AbbVie

- **Confirmed fact:** AbbVie's Part D brands are visible in current public Medicare formularies with meaningful control features, not just broad coverage.
- **Commercial interpretation:** Part D negotiation and formulary control are part of the same pricing conversation. If AbbVie is managing one part of the portfolio under CMS negotiation, the rest of the Part D portfolio cannot be treated as isolated.
- **Inference:** the more control-heavy the current Medicare formulary position, the more valuable a director is who can translate Part D behavior into a broader federal pricing doctrine.

8. Where Part B and Part D Intersect for AbbVie

1. Skyrizi is the clearest cross-benefit case

Confirmed fact: AbbVie's 2025 Form 10-K says Skyrizi is given as three induction doses via IV infusion for Crohn's disease and ulcerative colitis, followed by SC maintenance. The FDA label also shows both IV and SC presentations. AbbVie 2025 Form 10-K (<https://www.sec.gov/Archives/edgar/data/1551152/000155115226000008/abbv-20251231.htm>), SKYRIZI label (https://www.accessdata.fda.gov/drugsatfda_docs/label/2024/761105s032lbl.pdf)

Commercial interpretation: one brand must be managed across an infusion-center buy-and-bill pathway and a specialty-pharmacy / self-administered pathway. That means CMS coding, provider education, and plan formularies all matter at once.

Why this role cares: the person owns the translation between benefit design and launch/lifecycle execution. A dual-pathway brand like Skyrizi is exactly where that translation fails if nobody owns it.

2. Oncology regimen economics do not stop at the molecule

Confirmed fact: AbbVie's 2025 Form 10-K shows Venclexta, Imbruvica, and Elahere all as meaningful oncology brands, with Venclexta and Imbruvica oral and Elahere infused. AbbVie 2025 Form 10-K (<https://www.sec.gov/Archives/edgar/data/1551152/000155115226000008/abbv-20251231.htm>)

Commercial interpretation: a Part D oral agent can still be part of a clinic-based regimen, and a Part B infused agent can influence the economics of the total regimen. Reimbursement strategy therefore has to be regimen-aware, not molecule-only.

Why this role cares: the director must anticipate how a Part D decision changes clinic economics and how a Part B decision changes therapy sequencing, support, and site-of-care choices.

3. Botox shows how Part B negotiation spills into provider operations

Confirmed fact: CMS selected Botox/Botox Cosmetic for IPAY 2028, and AbbVie's 2025 Form 10-K confirms the selection. CMS 2028 selected-drug list (<https://www.cms.gov/files/document/factsheet-medicare-negotiation-selected-drug-list-ipay-2028.pdf>), AbbVie 2025 Form 10-K (<https://www.sec.gov/Archives/edgar/data/1551152/000155115226000008/abbv-20251231.htm>)

Commercial interpretation: Botox is multi-indication, billed by HCPCS unit, and supplied in single-dose vials. That makes negotiation, ASP, waste, and JZ/JW compliance part of one operational file. It is not a policy abstraction.

4. Part D negotiation lessons create enterprise doctrine

Confirmed fact: AbbVie already has 2026 and 2027 negotiated prices for Imbruvica, Vraylar, and Linzess. CMS 2026 negotiated prices (<https://www.cms.gov/files/document/fact-sheet-negotiated-prices-initial-price-applicability-year-2026.pdf>), CMS 2027 negotiated prices (<https://www.cms.gov/files/document/fact-sheet-negotiated-prices-ipay-2027.pdf>)

Commercial interpretation: the company can now compare one completed cycle, one active cycle, and one future cycle. That gives AbbVie enough data to build a real pricing-playbook doctrine for future Part B and Part D negotiations.

9. What AbbVie Likely Needs in the First 12 Months

Immediate fires

- Build a Botox 2028 negotiation war room with finance, legal, market access, medical, and brand leadership.
- Track CMS milestones, especially the June 1, 2026 initial offer window and the November 30, 2026 MFP publication date.
- Monitor ASP payment-limit updates for J0585, J2327, and J9063 each quarter.
- Prepare JZ/JW and discarded-drug guidance for single-dose vial products.

Medium-term build

- Create a unified Medicare heat map across Part B, Part D, and cross-benefit products.
- Build a standing CMS tracker covering negotiation, ASP, discarded-drug policy, and HCPCS/payment-limit changes.
- Refresh reimbursement support for Skyrizi, Elahere, and any future launch or label expansion that changes route, dose, or site of care.
- Formalize provider-society engagement plans for neurology, ophthalmology, oncology, GI, and infusion stakeholders.

Enterprise value-add

- Convert the Imbruvica, Vraylar, and Linzess negotiation experience into AbbVie-wide doctrine.
- Translate Part B lessons into a broader federal pricing and access strategy.
- Create a single operational language that legal, finance, commercial, and field teams can all use.
- Use Medicare policy as an early warning system for broader commercial payer behavior.

10. Joseph Stewart Fit Analysis

Why the bridge exists

The workspace materials show a candidate who already works at the intersection of reimbursement mechanics, payer access, provider economics, and policy translation. The strongest fit is not that Joseph has already run AbbVie’s exact programs. The strongest fit is that he already thinks in the same structure AbbVie needs here: policy to payment to behavior to commercial consequence. About page (app/about/page.tsx), Portfolio brief (portfolio_brief.md), Interview talking points (interview_talking_points.md)

AbbVie workstream	Why Joseph is relevant	How to phrase it in interview	Strongest bridge	Weaker bridge and honest fix
Medicare reimbursement strategy	Joseph’s background centers on reimbursement and market access, with current work spanning Medicare, Medicaid, commercial, launch planning, pricing models, and reimbursement risk.	“I translate reimbursement mechanics into commercial strategy, so teams can see how policy becomes access and revenue.”	Medicare fluency and access strategy.	He should not claim hands-on AbbVie buy-and-bill ownership; instead, position it as transferable reimbursement strategy.
CMS engagement	His portfolio shows policy translation and analytics, which is what CMS-facing engagement needs.	“I am comfortable turning policy into a clear, defensible position for CMS and for internal leadership.”	Policy-to-commercial translation.	Direct CMS negotiation execution is not supported by the resume facts; say that plainly.
Part D / formulary analytics	The About page explicitly says he focuses on Medicare Part D, formulary behavior, PBM dynamics, and healthcare data analysis using CMS SPUF and plan-level data.	“I have spent time on the plan side of the story, so I can connect formulary behavior to strategy.”	Part D depth and analytics.	Part B buy-and-bill depth is likely weaker; frame it as an area where he has strong adjacent expertise and is ready to deepen.
Launch and label-change planning	The portfolio materials emphasize launch readiness, site-of-care economics, coding visibility, and reimbursement architecture.	“I approach launch by asking whether the product is operationally absorbable, not just clinically differentiated.”	Launch readiness and reimbursement planning.	If asked about a specific AbbVie launch, avoid pretending ownership; stay at the framework level.
Provider and society engagement	The workspace materials show provider engagement, payer access, and commercialization strategy work.	“I treat provider engagement as a reimbursement execution problem, not a communications exercise.”	Provider economics and field usability.	Society-level oncology or neurology credibility should be framed as learning curve, not prior specialty ownership.
Biosimilars and pricing pressure	Otsuka, AAM, and biosimilars work give him a pricing and competition lens.	“I understand how pricing, substitution, and payer incentives shape uptake.”	Competitive and pricing dynamics.	Direct AbbVie franchise-specific experience is absent; that should be addressed as a fast ramp rather than a false equivalence.
Claims / adjudication workflow	CVS and PBM-adjacent experience support his understanding of real claims behavior and rejects/overrides.	“I have seen what happens when policy meets adjudication, which is where reimbursement strategies often break.”	Operational reimbursement realism.	CVS exposure is not the same as manufacturer Part B policy ownership; keep the scope honest.

Interview positioning guidance

- **Best framing:** “I am a reimbursement strategist who can connect Medicare policy, plan behavior, and provider economics to commercial action.”
- **Avoid:** claiming direct AbbVie ownership, direct CMS negotiation execution, or direct responsibility for an identical Part B portfolio if that is not supported by the resume.
- **Most credible bridge:** Medicare fluency plus payer/provider translation plus launch readiness.

11. Interview Answers

A. 90-second answer

Prompt: Why are you a fit for this AbbVie Director, Government Affairs - U.S. Reimbursement role?

I am a fit for this role because my work sits exactly at the intersection AbbVie needs: Medicare reimbursement, payer behavior, provider economics, and policy translation. My background includes manufacturer-facing consulting, Otsuka reimbursement and health policy work, biosimilars commercialization and reimbursement strategy, and real-world claims and PBM workflow exposure. That combination matters at AbbVie because the portfolio spans Part B products like Botox, Skyrizi IV, and Elahere, and Part D brands like Imbruvica, Vraylar, Linzess, Rinvoq, Qulipta, and Ubrelvy. I am strongest where the role needs someone to turn CMS policy into operational reimbursement strategy, and I am honest that the AbbVie-specific Part B depth would be a ramp, not a claim of prior ownership. That is the right fit for a director seat that needs judgment, not just policy monitoring. AbbVie 2025 Form 10-K

(<https://www.sec.gov/Archives/edgar/data/1551152/000155115226000008/abbv-20251231.htm>), About page (<app/about/page.tsx>)

B. 3-minute strategic answer

Prompt: How would you approach AbbVie's Medicare Part B reimbursement strategy?

I would start by treating AbbVie's Part B portfolio as a payment architecture problem, not a brand list. The first step is a product-by-product heat map that separates pure Part B products, Part D products, and cross-benefit products like Skyrizi. Then I would overlay the CMS payment environment: current ASP payment limits, discarded-drug policy, HCPCS coding, site-of-care economics, and the Medicare Drug Price Negotiation calendar. Botox would be the immediate priority because it is already selected for IPAY 2028 and it is operationally sensitive because it is a high-volume single-dose injectable with multi-indication use. From there I would refresh reimbursement playbooks for Skyrizi IV and Elahere, because they show the two classic Part B problems: mixed benefit pathways and buy-and-bill economics. I would also build a standing CMS tracker so AbbVie has one view of ASP, negotiation, and coding changes across the portfolio. The goal is to turn policy changes into predictable reimbursement actions before providers or plans create friction downstream. CMS 2028 selected-drug list

(<https://www.cms.gov/files/document/factsheet-medicare-negotiation-selected-drug-list-ipay-2028.pdf>), CMS Part B drugs overview

(<https://www.cms.gov/cms-guide-medical-technology-companies-and-other-interested-parties/payment/part-b-drugs>), AbbVie 2025 Form 10-K

(<https://www.sec.gov/Archives/edgar/data/1551152/000155115226000008/abbv-20251231.htm>)

C. Product-specific answer

Prompt: How do you think about AbbVie's Part B and Part D portfolio intersection?

I think about it at the product-and-regimen level. Skyrizi is the cleanest example because it starts with IV induction for Crohn's disease and ulcerative colitis and then moves into SC maintenance. That means the same brand crosses from provider-administered reimbursement to specialty-pharmacy or self-administered reimbursement. Venclexta is another example: it is a Part D oral oncology product, but in practice it lives inside clinic regimens that affect provider economics and site-of-care behavior. Botox is the opposite direction: it is a Part B therapeutic product now selected for negotiation, but the wider AbbVie portfolio also includes Part D brands whose formulary management will influence how AbbVie thinks about pricing, access, and policy spillover across the enterprise. The commercial point is that AbbVie cannot manage Medicare as two separate silos. The role needs somebody who can see the channels together and design the reimbursement strategy accordingly. AbbVie 2025 Form 10-K (<https://www.sec.gov/Archives/edgar/data/1551152/000155115226000008/abbv-20251231.htm>), SKYRIZI label (https://www.accessdata.fda.gov/drugsatfda_docs/label/2024/761105s032lbl.pdf), CMS 2028 selected-drug list (<https://www.cms.gov/files/document/factsheet-medicare-negotiation-selected-drug-list-ipay-2028.pdf>)

D. IRA answer

Prompt: What does AbbVie need to be thinking about as Medicare negotiation expands into Part B?

AbbVie needs to think about negotiation as an operational reimbursement event, not only a price event. With Part B in the program, the price flows into ASP-based payment-limit logic, discarded-drug behavior, site-of-care economics, and provider messaging. That is different from a Part D-only negotiation, where the immediate operational issues are mostly formulary and dispensing. For AbbVie, the lesson from Imbruvica, Vraylar, and Linzess is that the company should already have a doctrine for how to respond when CMS selects a product. For Botox specifically, the key is to be ready before the MFP is public because providers will need a clean explanation of how the policy affects coding, payment, and waste. The strongest companies here will not just react to CMS. They will enter the cycle with a reimbursement playbook that can absorb the policy change without losing provider confidence. CMS 2026 negotiated prices

(<https://www.cms.gov/files/document/fact-sheet-negotiated-prices-initial-price-applicability-year-2026.pdf>), CMS 2027 negotiated prices

(<https://www.cms.gov/files/document/fact-sheet-negotiated-prices-ipay-2027.pdf>), IPAY 2028 final guidance (<https://www.cms.gov/files/document/ipay-2028-final-guidance.pdf>)

E. Day-one priorities answer

Prompt: What would your first 90 days look like?

In the first 90 days I would do four things. First, build a Medicare portfolio heat map that separates Part B, Part D, and cross-benefit products, with Botox, Skyrizi, and Elahere flagged as the highest Part B priorities. Second, stand up a Botox 2028 workflow that tracks CMS milestones, payment-limit exposure, discarded-drug implications, and provider messaging. Third, refresh the reimbursement view for the highest-risk Part D brands so AbbVie can see where Part D management is likely to spill into broader enterprise pricing or access strategy. Fourth, create a standing CMS and policy tracker that connects negotiation, ASP, coding, and coverage into one working document for leadership. My goal would be to give AbbVie a common reimbursement language quickly, so the team is not reacting issue by issue when the next CMS milestone lands. CMS 2028 selected-drug list (<https://www.cms.gov/files/document/factsheet-medicare-negotiation-selected-drug-list-ipay-2028.pdf>), ASP Pricing Files (<https://www.cms.gov/medicare/payment/part-b-drugs/asp-pricing-files>), AbbVie 2025 Form 10-K (<https://www.sec.gov/Archives/edgar/data/1551152/000155115226000008/abbv-20251231.htm>)

12. Source Appendix

Primary sources

- AbbVie 2025 Form 10-K (<https://www.sec.gov/Archives/edgar/data/1551152/000155115226000008/abbv-20251231.htm>)
- AbbVie 2025 Full-Year and Fourth-Quarter Financial Results (<https://news.abbvie.com/2026-02-04-AbbVie-Reports-Full-Year-and-Fourth-Quarter-2025-Financial-Results>)
- CMS 2026 negotiated prices fact sheet (<https://www.cms.gov/files/document/fact-sheet-negotiated-prices-initial-price-applicability-year-2026.pdf>)
- CMS 2027 negotiated prices fact sheet (<https://www.cms.gov/files/document/fact-sheet-negotiated-prices-ipay-2027.pdf>)
- CMS 2028 selected-drug list fact sheet (<https://www.cms.gov/files/document/factsheet-medicare-negotiation-selected-drug-list-ipay-2028.pdf>)
- CMS third-cycle manufacturer participation fact sheet (<https://www.cms.gov/newsroom/fact-sheets/cms-announces-manufacturer-participation-third-cycle-medicare-drug-price-negotiation>)
- CMS IPAY 2028 final guidance (<https://www.cms.gov/files/document/ipay-2028-final-guidance.pdf>)
- CMS ASP Pricing Files (<https://www.cms.gov/medicare/payment/part-b-drugs/asp-pricing-files>)
- CMS Part B Drugs overview (<https://www.cms.gov/cms-guide-medical-technology-companies-and-other-interested-parties/payment/part-b-drugs>)
- CMS CY 2026 PFS final rule fact sheet (<https://www.cms.gov/newsroom/fact-sheets/calendar-year-cy-2026-medicare-physician-fee-schedule-final-rule-cms-1832-f>)
- CMS discarded-drug policy article (<https://www.cms.gov/medicare-coverage-database/view/article.aspx?CptHcpcsCode=j1561&articleId=52855>)
- FDA BOTOX label (https://www.accessdata.fda.gov/drugsatfda_docs/label/2023/103000s5327lbl.pdf)
- FDA SKYRIZI label (https://www.accessdata.fda.gov/drugsatfda_docs/label/2024/761105s032lbl.pdf)
- FDA ELAHERE label (https://www.accessdata.fda.gov/drugsatfda_docs/label/2022/761310s000lbl.pdf)
- MetroPlusHealth CY 2026 Medicare formulary PDF (https://metroplus.org/wp-content/uploads/2026/01/H0423_MEM26_3567_C-09192025-MPH-formulary_Advantage_assembled_01202026.pdf)
- CMS ASP payment-limit ZIP for April 2026 (<https://www.cms.gov/files/zip/april-2026-medicare-part-b-payment-limit-files-03-30-2026-final-file.zip>)

Internal workspace sources

- About page (app/about/page.tsx)
- Portfolio brief (portfolio_brief.md)
- Portfolio master (joseph_market_access_portfolio_master.md)
- Interview talking points (interview_talking_points.md)
- AbbVie draft notes (Abbvie.md)